Gynecological clinical case presentation

Neha Makkithaya

Name : Mrs XYZ

Age : 68 years

Residence : Mangalore

Occupation : Housewife

Education : 3rd standard

Socioeconomic status : Lower middle class

Parity index : $P_3L_3A_0$

DOA : 29/02/20

DOE : 06/03/20

Chief complaints

- Sensation of something protruding out of the vagina since 8 years

History of present illness

- Patient was apparently in normal state of health 8 years ago after which she presents with pelvic heaviness and mass per vagina.
- The mass increases in size after prolonged exertion like lifting and standing.
- It reduces while lying down or when the bulge is pushed inside.

- No h/o bleeding PV.
- No h/o stress incontinence
- No h/o difficulty in initiating micturition.
- No h/o increased frequency of urination.
- No h/o burning micturition.
- No h/o bowel disturbance.
- No h/o pain abdomen.
- No h/o mass per abdomen
- No h/o vaginal discharge.
- No h/o chronic cough, chronic respiratory disease.
- No h/o pelvic trauma.
- No h/o connective tissue disorders.

Menstrual history

- Menopause attained 20 years ago
- Menarche attained at 14 years of age
- Regular 28-30 days of cycle
- 3-4 days of flow
- Used 2-3 cloths per day
- No passage of clots
- No h/o dysmenorrhea

Obstetric history

1. **First pregnancy**

- It was a male child who is 42 years of age now.
- Delivery was done at term.
- It was a normal vaginal delivery done at home.
- Baby was average size as compared to the other two offsprings
- No h/o complications during labour.

2. Second pregnancy

- It was a female child who is 38 years of age now.
- Delivery was done at term.
- It was a normal vaginal delivery done at hospital.
- Baby weighed 2.5 kg at birth and was breastfed.
- No h/o prolonged labour.
- No h/o instruments used
- No h/o complications during labour.

3. Third pregnancy

- It was a female child who is 32 years of age now.
- Delivery was done at term.
- It was a normal vaginal delivery done at hospital.
- Baby weighed 3 kg at birth and was breastfed.
- No h/o prolonged labour.
- No h/o instruments used
- No h/o complications during labour.

Past history

- She is a known hypertensive since 30 years and is on antihypertensives.
- She is a known diabetic since 8 years and is on oral hypoglycemic drugs for the same.
- Not known case of bronchial asthma, tuberculosis and epilepsy.
- No previous surgeries done.
- No previous admissions.
- No known allergies.

Family history

- No h/o similar complaints in the family
- No h/o hypertension, diabetes, tuberculosis, bronchial asthma and epilepsy in the family.

Personal history

- Consumes a mixed diet
- Appetite is normal
- Bowel and Bladder habits are normal
- Adequate sleep
- No h/o any addictions

Summary

- 68 year old post menopausal, multiparous (Parity index – P3L3A0) hypertensive and diabetic woman with history of normal vaginal delivery conducted at home came with chief complaints of sensation of something protruding from vagina since 8 years is probably suffering from uterine prolapse.
- Hence, I would like to examine the abdomen and a per vaginal examination.

General physical examination

- Patient is conscious, cooperative and well oriented to time, place and person.
- She is moderately built and nourished.
- Weight – 74 kg Height – 160 cm
  BMI – 28.1kg/m^2
- Pallor present
- No icterus
- No cyanosis
- No clubbing
- No generalised lymphadenopathy
- No generalised edema

- Vitals

- Pulse rate – 70 beats/min, regular rhythm, normal volume, normal character, no vessel wall thickening and no radiofemoral delay
- Blood pressure – 130/80 mmHg. Taken from left hand in sitting position
- Respiratory rate – 18 cycles/min, thoracoabdominal type
- Temperature - Afebrile

- Thyroid – normal
- Breast – normal
- Spine – normal

Abdominal examination

I) **Inspection**

- Abdomen is slightly distended
- Position of umbilicus is central and inverted
- Corresponding areas move equally with respiration
- No scars, sinuses or dilated veins seen
- Hernial orifices are intact

II) **Palpation**

- No local rise in temperature
- No tenderness.
- Abdomen is soft
- No palpable mass
- No organomegaly

III) Percussion

- Tympanic note heard
- Liver dullness heard at the right 5th intercostal space

IV) Auscultation

- Bowel sounds heard

Per vaginal examination

I) **Inspection**

- 6 x 8 cm size mass protruding out of the interoitus
- Rugosities in the anterior vaginal wall are absent.
- Leading end - cervix is seen
- Appears to bulge on straining
- Cough impulse is positive
- No ulceration or bleeding over the surface
- No areas of hyperpigmentation seen.

II) **Palpation**

- Mass protruding can be reduced manually
- Cannot get above the mass
- No cervical motion tenderness
- No adnexal tenderness
- No bleeding on touch

Systemic examination

Cardiovascular system

S1, S2 sounds heard, no murmurs heard

Respiratory system

Normal vesicular sounds heard

No added sounds

Central nervous system

No focal neurological deficit

Provisional diagnosis

- 68 year old post menopausal, multiparous, hypertensive and diabetic woman who came with chief complaints mass per vagina which worsens on physical exertion and reduces on manual manipulation and on per vaginal examination a reducible mass is present outside the interoitus, one cannot get above the mass is probably suffering from a 3rd degree uterine prolapse.